



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : نيللى عبداللطيف عبداللطيف السيد

المحافظة : الجيزة الحي / المدينة : الهرم

رقم البطاقة : 27709190104126 رقم التليفون : 01275221878

العمر (تاريخ الميلاد) : 19-09-1977 المهنة : ربه منزل

العنوان : فيصل

تاريخ تسجيل الإستمارة : 29-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي بيمبروليزوماب وريدي [14335] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: نيلي عبداللطيف السيد
National ID: 27709190104126



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Pembrolizumab IV

الوقت :

-- --:--

التاريخ :

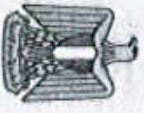
05-07-2026

الطبيب المعالج :
د/شيماء لاشين


 جمهورية مصر العربية
 بطاقة تحقيق الشخصية
 نبلى
 عبد الطيف عبد الطيف السيد
 بل الصفاى المرومىة الرصن لصل
 الهمم - الورد
 ٢٧٧.٩١٩.٠١.٤١٢٦
 IAA433000

Signature


 ٢٠١٩/٠٥ ٢٧٧.٩١٩.٠١.٤١٢٦
 بلون عل
 انلى
 صلاخ مصطفى عبد المصطوف احمد
 البلققة سارية حتى ٢٠٢١/٥/٢١



مخرج
 ذى
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 مخرج

Handwritten note

Patient Name : الأستاذة / نبيلة عبد اللطيف عبد اللطيف
Patient No. : 27709190104126
Path No. : 582/26
Sex : Female
Age : 48 Y
Referring Doc :

Request No. : 13005263
Request Date : 02/03/2026
Verification Date : 08/03/2026
Financial Category : جوسف روسي - مبادره
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast mass, (BIRADs 4c).

Nature of Specimen: Tru-cut biopsy, collected on 2/3/2026, at 9:35 am, Cold ischemia time: 20 seconds.

Gross Examination:

- Fragmented grey white tissue cores, ranging in size between 0.3 cm & 1.5 cm long, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Perineural invasion: Absent.
- * Angioinvasion: Absent.
- * Necrosis: Focal.

Diagnosis:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 3.
- For further IHC (ER, PR, Her2, Ki67).

Signature

Dr. Walaa Ibrahim Ghonaim.

Verified by :

Dr.Duaa Mohamed

Printing Date : 08/04/2026 11:17:57

Page 1 of 1

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name: نبلى عبداللطيف عبداللطيف السيد		
MR. : 9935		
Detailed Medical Report		
		السيد
تحية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمناظرة وجد انه The patient has a history of hypertension, is married with four offspring, and was diagnosed with invasive ductal carcinoma (IDC) of the left breast measuring 5 × 3.3 cm with involvement of more than three axillary lymph nodes; the tumor is triple-negative on staging work-up. Multidisciplinary discussion in June 2026 concluded that, given cT3 N1+ M0 triple-negative breast cancer (TNBC), the treatment plan should include port-a-cath insertion, clip placement, neoadjuvant chemotherapy (NACT) with weekly Taxol and carboplatin (planned for 12 cycles) followed by four cycles of AC, concomitant pembrolizumab (Keytruda) per the KEYNOTE-522 protocol, BRCA testing, and consideration of left breast wide local excision with axillary lymph-node management based on response. On 9 August 2026 the patient received week 1 (W1) of Taxol 75 mg/m² and carboplatin AUC 2, with planned continuation of weekly treatment; pembrolizumab 200 mg was scheduled to start after laboratory results (height 153 cm, weight 62 kg). The same regimen was ordered for weeks 2 (W2) and 3 (W3) through the pharmacy. On 16 August 2026 the clinical pharmacy note recorded the patient's weight as 62 kg, height 158 cm (body surface area 1.62 m²), and confirmed dosing for week 2 (W2) Taxol 122 mg and carboplatin AUC 2. On 23 August 2026 the pharmacy documented continuation of the regimen with week 3 (W3) Taxol and carboplatin AUC 2, along with the first dose of Keytruda (C1). On 30 August 2026 the medical oncology note described the left breast cancer as cT3 N1 M0 IDC, triple-negative, Ki-67 80 %, estrogen receptor 67 %. Imaging showed a right hepatic cyst, a few tiny pulmonary nodules, and a right breast BI-RADS 3 lesion. The patient received week 4 (W4) Taxol 122 mg, carboplatin AUC 2, and one dose of pegfilgrastim (Neupogen) in preparation for clip insertion. On 6 September 2026 the internal medicine note reiterated the multidisciplinary plan (port-a-cath, clip insertion, NACT Taxol/Carboplatin ×12 followed by AC ×4 with concurrent Keytruda per KEYNOTE-522, BRCA testing, and surgical considerations). It also referenced upcoming treatment weeks: W5 with taxane-carboplatin, W6 with taxane-carboplatin plus Keytruda, and W7 in preparation for surgery. The patient has completed four weeks (W4) of the taxane-carboplatin neoadjuvant regimen to date.		
تحريرا في ,		
	التاريخ : 19-09-2026	الوقت :
	نهي سعيد علي محمد	الطبيب المعالج :
يعتمد ,		

Patient Name	: الأستاذة / نيللي عبد اللطيف عبد اللطيف	Request No.	: 13005263
Patient No.	: 27709190104126	Request Date	: 02/03/2026
Path No.	: 582/26	Verification Date	: 08/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 48 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass, (BIRADs 4c) diagnosed "Invasive duct carcinoma grade 3".
Nature of Specimen: Tru-cut biopsy, collected on 2/3/2026, at 9:35 am, Cold ischemia time: 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8)
- PR: Negative nuclear reaction in tumor cells (score 0/8)
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (Ultra-low, score 0+).
- Ki67%: 80%.

Conclusion:

Left breast mass, Tru-cut biopsy:

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Ultra low (score 0+).
- Ki67%: 80%.

*** Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Verified by :

Dr.Duaa Mohamed

Printing Date : 08/04/2026 11:17:45

Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name الأستاذ / نبلي عبد اللطيف عبد اللطيف
Patient No. 27709190104126
Path No. 582/26
Sex Female
Age 48 Y
Referring Doc.

Request No. 13005263
Request Date 02/03/2026
Verification Date 08/03/2026
Financial Category جوسف روسي - مبادره
Department Out Patient

Registered by Mrs. Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0 (Ultra-low): as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghonaim

Verified by :
Dr. Duaa Mohamed

Printing Date : 08/04/2026 11:17:45

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

مركز المريض بالإنجليزية: 93582 كود المريض: 93582	اسم المريض بالإنجليزية: 2026/09/01 01:04:29 PM تاريخ التقرير: 2026/09/01 01:04:29 PM
السن: ٤٨ سنة ١١ شهر	

Ultrasound Guided clip insertion:

Clinical summary: Pathologically proven Left breast carcinoma for clip insertion.

Technique:

- Under complete sterile conditions one metallic clip is inserted within the center of large malignant mass lesion at the left breast and measures today about 4 x 2 cm and located at 2 o'clock position.
- This was confirmed by mammography CC & ML views.

*Best Regards
Dr. Iman Adel
Breast Imaging consultant
& Breast Intervention*

 		مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية	
MDT FORM		07/01/2026	
Name:	نبلى عبداللطيف عبداللطيف السيد	MRN:	9935
Age:	49 Y	ID:	27709190104126
Menopausal Sta	pre	Phone:	01275221878
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	HTN	Reason Of MDT:	new

Clinical Data:

BX:	IDC G 3	HER2:	0	KI67:	80%
ER:	0/8	PR:	0/8		

Sonomammography:

UOQ at 1-2 o'clock oval shaped indistinct heterogeneous hypoechoic vascular palpable lesion measuring about 5X3.3 cm (8 cm from the nipple, abutting overlying skin) ? Normal skin and nipple/areolar complex.

- More than three axillary level I LNs showing non-uniform cortical thickening reaching 0.39 cm and effaced hilum.

Impression:

- ? Left breast (BIRADS 4c): UOQ highly suspicious lesion for US guided biopsy. ? Right breast (BIRADS 3) UOQ benign looking lesion for follow up.

Mets Workup:

CT:

- Few tiny right pulmonary nodules. • Right hepatic lobe small lesion likely cyst.
- Bulky left suprarenal gland. • Posterior uterine fibroid.

bone scan : free
ECHO normal

History:

Recommendations:

MDT Outcome & Plan

In view of having cT3 N1+ M0, TNBC, for:

- 1.Portacath insertion
- 2.Clip insertion
- 3.Start NACT Taxol/Carbo x12 followed by AC x4 with concomitant \Keytruda "Keynote 522"
- 4.BRCA testing
- 5.Consider Lt WLE & axillary LN management according to response.

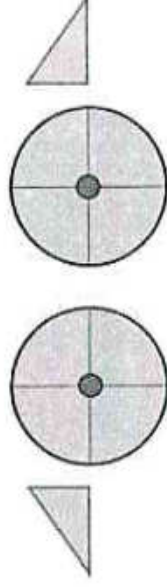
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MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					

1/1/2026
1/7/2026

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TNM

 		مبادرة صحة المرأة المصرية لدعم صحة المرأة المصرية	
MDT FORM		07/01/2026	
Name:	نبلى عبداللطيف عبداللطيف السيد	MRN:	9935
Age:	49 Y	ID:	27709190104126
Menopausal Sta	pre	Phone:	01275221878
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	HTN	Reason Of MDT:	new

Clinical Data:

BX:	IDC G 3
ER: 0/8	PR: 0/8 HER2: 0 KI67: 80%

Sonomammography:

UOQ at 1-2 o'clock oval shaped indistinct heterogeneous hypoechoic vascular palpable lesion measuring about 5X3.3 cm (8 cm from the nipple, abutting overlying skin) ? Normal skin and nipple/areolar complex.

- More than three axillary level I LNs showing non-uniform cortical thickening reaching 0.39 cm and effaced hilum.

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- Bulky left suprarenal gland. • Posterior uterine fibroid.

bone scan : free
ECHO normal

History:

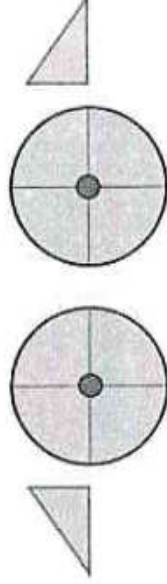
Recommendations:

MDT Outcome & Plan

In view of having cT3 N1+ M0, TNBC, for:

- 1.Portacath insertion
- 2.Clip insertion
- 3.Start NACT Taxol/Carbo x12 followed by AC x4 with concomitant \Keytruda "Keynote 522"
- 4.BRCA testing
- 5.Consider Lt WLE & axillary LN management according to response.

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SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
<i>[Signature]</i>	<i>[Signature]</i>				

1/7/2026
1/7/2026

جمهورية مصر العربية
وزارة الداخلية
نيللي
عبداللطيف عبداللطيف السيد
١٣ ش الصفا والعروسة مدينة الرحمن فيصل
الهرم - الجيزة
٢٧٧.٩١٩.٠١.٤١٢٦
LT5732449





جمهورية مصر العربية
بشارة محمد النعمانية

نيللي

عبد المظفر عبد المظفر السيد

من الصلوات المبرورة - مدينة الوحدانية - فرع

المرم - الجيزة

٢٧٧٠٩١٩٠١٠٤١٢٦

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٢٠١٩/٠٥

٢٧٧٠٩١٩٠١٠٤١٢٦

بولن عل



التي

مصلحة

صلاح مصطفى عبد المصطفى احمد

البطاقة مادية حتى ٢٠٢١/٠٥/٢٧

١٥٧٥٤٤١٨٧٧
١٤٧٤٥٥٢٣٩٩

Patient ID: 93581	Patient Name English: nilli Abdallah Abdallah
المرضى: 93581	2026/02/25 11:05:26 AM :Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography

Clinical summary: Left breast lump.

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Left breast

- Scattered fibro glandular densities (ACR b).
- UOQ posterior 1/3 partially circumscribed partially indistinct oval shaped hyperdense lesion.
- Ultrasound demonstrated UOQ at 1-2 o'clock oval shaped indistinct heterogeneous hypoechoic vascular palpable lesion measuring about 5X3.3 cm (8 cm from the nipple, abutting overlying skin).
- Normal skin and nipple/areolar complex.
- More than three axillary level 1 LNs showing non-uniform cortical thickening reaching 0.39 cm and effaced hilum.

Right breast

- Scattered fibro glandular densities (ACR b).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated UOQ at 11 o'clock circumscribed oval shaped iso to hypoechoic small lesion measuring 0.9X0.4 cm.
- Few small cysts.
- Normal skin and nipple/areolar complex.
- Preserved axillary LNs.

Impression:

- Left breast (BIRADS 4c): UOQ highly suspicious lesion for US guided biopsy.
- Right breast (BIRADS 3) UOQ benign looking lesion for follow up.

Dr. Amira Shaheen, MD

⑦

Patient Name : الأستاذ / نبالي عبد اللطيف عبد اللطيف
Patient No. : 27709190104126
Path No. : 582/26
Sex : Female
Age : 48 Y
Refereing Doc :

Request No. : 13005263
Request Date : 02/03/2026
Verification Date : 08/03/2026
جورستاف روسي - ميلاد :
Financial Category : Out Patient
Department :
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Left breast mass, (BIRADs 4c).

Nature of Specimen: Tru-cut biopsy, collected on 2/3/2026, at 9:35 am, Cold ischemia time: 20 seconds.

Gross Examination:

- Fragmented grey white tissue cores, ranging in size between 0.3 cm & 1.5 cm long, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Perineural invasion: Absent.
- * Angioinvasion: Absent.
- * Necrosis: Focal.

Diagnosis:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 3.
- For further IHC (ER, PR, Her2, Ki67).

Signature

Dr. Walaa Ibrahim Ghonaim.

Verified by :

Dr. Duaa Mohamed

2



Patient Name : الأستاذ / نبلي عبد الطيف عبد الطيف
Patient No. : 27709190104126
Path No. : 582/26
Sex : Female
Age : 48 Y
Refereing Doc. :

Request No. : 13005263
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Financial Category : جراحات روسي - مبلرة
Department : Out Patient
IHC No. :
Registered by : Mrs. Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass, (BIRADs 4c) diagnosed "Invasive duct carcinoma grade 3".
Nature of Specimen: Tru-cut biopsy, collected on 2/3/2026, at 9:35 am, Cold ischemia time: 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP-1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Negative nuclear reaction in tumor cells (score 0/8)
- PR: Negative nuclear reaction in tumor cells (score 0/8)
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (Ultra-low, score 0+).
- Ki67%: 80%.

Conclusion:

Left breast mass, Tru-cut biopsy:

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Ultra low (score 0+).
- Ki67%: 80%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Verified by :

Dr. Duaa Mohamed

Printing Date : 29/03/2026 12:21:16

Page 1 of 2

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Name : Nelly Abdul Latif Abdul Latif El-Sayed**Date : 20-06-2026 08:30****Branch : Maadi****Medical No. : 397774****Ref.Physician Prof : Goustav Roussey Egypt****تعلق مستشفيات : Ins.Company**

Whole-Body Bone Scintigraphy Report

Clinical History

- A 48-year-old female presenting with history of left breast carcinoma.
- The current whole-body bone scintigraphy is performed for initial staging and evaluation of potential skeletal metastases.

Technical Procedure: Following the intravenous administration of Tc-99m MDP, routine static whole-body anterior and posterior planar images were acquired approximately 3 hours post-injection.

Scintigraphic Findings

- **Skeleton:** Normal, symmetrical radiotracer distribution throughout the entire axial and appendicular skeleton. No focal areas of abnormal increased or decreased osteoblastic activity identified within the skull, spine, ribs, sternum, pelvis, or extremities.
- **Soft Tissues:** Normal physiological renal excretion and urinary bladder activity. No abnormal extraosseous tracer accumulation.

Conclusion

- **Normal Whole-Body Bone Scan.** No scintigraphic evidence of active osteoblastic skeletal metastases at the present time.
- **Recommendation:** Follow-up imaging as per oncological protocol.

Dr. Alaa Nagi, MD
Nuclear Medicine

6

ECHO REPORT

حالتى عبر الطيف عبر القلب

NAME :

DATE: 8 / 6 / 2026

ID NUMBER :

SEX :

AGE:

EF	67.1	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.7	3.5-5.8cm	AO.R	2.7	3-3.7cm
LVEDS		2-4cm	LA	3.2	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions

normal

LV Wall Thickness

normal

LV Diastolic Function

normal

SWMA

sub-A

LA and Ao Root Dimensions

normal

Mitral Valve

normal

Aortic Valve

normal

Tricuspid Valve

normal TR

Pulmonary Valve

normal

Right Side and Systolic Function

normal

Intracardiac Masses and Effusion

normal

DR.

Dr. Hisham

(Signature)

Patient's Name: Nilly Abdellatif Abdellatif
Date: 2026-04-20
Referred by: Prof.Dr. CAPwithC
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

A known case of left breast cancer for metastatic workup

I- CHEST:

- The left breast showed UOQ soft tissue mass lesion with central necrosis measures 5.1x4.9x5.7 cm in its maximum AP,TS and CC dimensions respectively, the lesion is seen resting/ inseparable from underlying pectoralis muscle. Associated with multiple enlarged left axillary lymph nodes, the largest measures about 19.9x19.6 mm **for sono-mammography.**
- Prominent right axillary lymph nodes, the largest measures about 9.6mm
- Few tiny right pulmonary nodules, the largest is seen in right upper anterior segment measures about 3.5mm
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. Right hepatic lobe segment VIII showed tiny non enhanced focal lesion measures about 5 mm
- No dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen (splenules, the largest measures about 1.3 cm) , pancreas, right suprarenal gland, aorta and IVC. Bulky left suprarenal gland.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.

Patient's Name: Nilly Abdellatif Abdellatif
Date: 2026-04-20
Referred by: Prof.Dr. CAPwithC
Hospital: Goustave Roussey International Egypt

- Bulky uterus with posterior uterine wall fibroid measures about 2x1.8 cm
- left ovarian cyst measures about 1.5 cm likely functional
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- **The examined skeleton** no worrisome lytic or sclerotic lesions.

Opinion:

- Few tiny right pulmonary nodules.
- Right hepatic lobe small lesion likely cyst.
- Bulky left suprarenal gland.
- Posterior uterine fibroid.

Dr.Nourhan Ramzy

Dr.Nourhan Ramzy

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